

PERSONA TEST LOG — SIGUE MX

Week 5 · Adversarial persona test

Persona

María, 55, informal worker in Mexico City. She does not have a stable employer-based health plan. She receives a simulated screening result that says her risk is elevated. She has limited time, is worried about cost, and is not sure whether the result means she already has a disease. She can use WhatsApp and a phone but does not want a complicated process.

What we tested

The test focuses on the critical transition from detection to action: can a person with limited time, money and system knowledge understand the result and know what to do next?

Where the persona gets lost

1. She may read a risk label as a diagnosis. 2. She may not know whether she should go immediately or wait. 3. A generic 'see a doctor' instruction does not solve navigation. 4. If the first route is inaccessible, she may stop. 5. A list of providers without context creates another search task.

What was fixed

The product separates **Detection** → **Interpretation** → **Action**. It uses explicit language that screening is not diagnosis; gives a short explanation of what the result means and does not mean; asks for practical constraints; prioritizes a small number of route types instead of a giant directory; adds appointment preparation; and includes a follow-up state for 'couldn't access' with a barrier so the system can propose another route.

Acceptance criteria after the fix

The persona can answer: 'What does my result mean?', 'What should I do next?', 'What do I need for the appointment?', and 'What can I do if I cannot use the first route?' without the product diagnosing her or inventing real-world availability.

Adversarial conclusion

The strongest failure mode is not inaccurate AI; it is abandonment after a technically correct result. The MVP therefore treats follow-through as a product requirement, not a postscript.